

Fictive Chat

Tobacco cessation — fictive patient chats

Illustrative dialogues between a person who smokes (or is trying to quit) and a health assistant. Derived from the QuitNowTXT message library (Annex 2 of WHO *Be He@lthy Be Mobile — mTobaccoCessation handbook*) and the *Gambia cessation clinical guidelines* (2016). All names and specifics are invented; numbers and recommendations are illustrative.

Chat 1 — Thinking about quitting

Ebrima: I smoke about 15 a day. My wife wants me to quit. I don't know if I can.

That's a normal feeling. Almost everyone who has ever quit had that exact thought. The fact that you're even thinking about it means you've already done the hardest part — deciding to consider it.

Let me ask you two things: do you want to quit for yourself as well, or just to keep peace at home? And how soon after waking do you smoke your first cigarette of the day?

Ebrima: Within 30 minutes of waking. And, honestly, I'd be glad to quit — I can feel I get out of breath climbing stairs now.

That early-morning cigarette tells me your nicotine addiction is in the moderate-to-strong range, which means medication (like nicotine patches or gum) would probably help a lot. The breathlessness you mentioned is worth taking seriously — it usually improves within weeks of quitting.

Here's how I'd suggest starting:

1. Pick a **Quit Day** in the next 1-2 weeks. Choose a day with low stress.
2. Between now and then, **identify your triggers** — what times of day and which situations make you reach for one.
3. Tell your wife the date, and ask her to support you (not nag — there is a difference).
4. Talk to a clinic about nicotine replacement (patch or gum). It roughly doubles your chance of staying smoke-free.

Want me to help you build your trigger list and a substitute for each?

Chat 2 — Quit Day morning

Mariama: Today is my Quit Day. I'm already wanting one and it's only 7am.

You're stronger than you think. The first morning is the worst — you've trained your body to expect that wake-up cigarette for years, and it's asking for it now. **Cravings feel huge but they pass in 5-10 minutes.**

Try right now: - Pour yourself a glass of cold water. - Take 10 slow, deep breaths. - Step outside for 2 minutes if you can. - Do something with your hands — wash dishes, fold a shirt, anything.

Have you eaten breakfast? Hunger sharpens cravings.

Mariama: Just had tea.

Tea is one of the most common pairings with cigarettes — if your morning tea has always come with a smoke, the tea by itself will trigger the craving for a while. Try changing the ritual: switch to coffee, switch to a different mug, drink the tea standing up or in a different room. Anything that breaks the old script.

Throughout today: keep busy, keep moving, keep your reasons-for-quitting list nearby. You can do this — millions of ex-smokers have done it, one day at a time.

Chat 3 — A slip on day 5

Lamin: I caved. I had a cigarette with my friend last night. I feel like I ruined everything.

You didn't ruin anything. One cigarette is a slip, not a relapse. The 4 days you were smoke-free still count. **The question now isn't whether you slipped — it's what you do next.**

Two questions: 1. What was happening right before you smoked? Who were you with, what were you feeling, where were you? 2. How many cigarettes did you have — one, or the whole pack?

Lamin: Just one. I was with my friend who smokes and he offered me one. After a few beers I just took it.

Two big triggers there: a smoking friend, and alcohol. Both are very common relapse triggers, especially together. That doesn't mean you need to avoid your friend forever — it means for the next few weeks, when you're with him, you either don't drink, or you ask him to not offer.

Here's what to do today:

- **Don't smoke today.** That's all you need to commit to right now.
- **Recommit to your Quit Day.** Pick this morning as a fresh start.
- **Reach out** to whoever is supporting you — tell them you slipped, what triggered it, and what you're going to do differently.

Most ex-smokers had several slips before they quit for good. You're not back to square one — you have more information about your triggers than you did a week ago. That's progress.

Chat 4 — Asking about hookah / shisha

Aminata: I don't smoke cigarettes but I do shisha with friends on weekends. That's safer, right?

Unfortunately, no. A typical shisha session lasts about 45 minutes to an hour, and during that time you inhale about as much smoke as 100 cigarettes worth — sometimes more. Plus you get extra carbon monoxide from the charcoal.

The risks are the same family as cigarettes: heart disease, lung disease, mouth and throat cancer. The water in the pipe does not filter out the harmful chemicals — that's a common misconception.

Aminata: But I only do it twice a month.

Twice a month is much better than weekly. The damage from any tobacco exposure adds up over years, so reducing how often you do it matters. The lowest-risk option is to stop, but if you're not ready to do that yet, two things to think about:

- Could you go to the same social gatherings without the shisha — drink juice or tea with your friends instead?
- Could you make it once a month for the next few months, and see if you can let it go from there?

There's no need to be all-or-nothing today.

Chat 5 — Withdrawal symptoms

Ousman: I'm on day 3 and I want to bite someone's head off. Is this normal?

Completely normal. **Irritability peaks around day 2-3 and starts easing by the end of week 1.** This is your brain rebalancing without nicotine. Other things you might be feeling: trouble sleeping, difficulty focusing, hunger, mild headaches.

These are signs your body is healing, not signs that quitting is wrong for you.

A few things that help in the next 4-5 days: - **Drink lots of water** — it helps your body clear the nicotine and reduces the headaches. - **Move your body** — even a 15-minute walk lowers irritability noticeably. - **Sleep early** if you can — sleep is your best ally this week. - **Avoid coffee** if it's making you more jittery. Switch to herbal tea or water for a couple of weeks. - **Tell people what you're going through** so they don't take the irritability personally.

Have you been using nicotine patches or gum? If not, and the cravings or irritability are unbearable, talk to a pharmacist or clinic this week. It is much more effective to use medication than to rely on willpower alone.

Chat 6 — Relapsed and back

Binta: I quit for 3 months but I've been smoking again for 6 weeks. I'm so disappointed in myself.

Three smoke-free months is a real achievement, not something to throw away. Many people who eventually quit for good went through several cycles of quitting and relapsing first. That's not failure — that's how this works for most people.

Let's think practically. What happened around the time you started smoking again? Was it a stressful event, a social situation, a particular trigger that came up?

Binta: My father was sick and I started just to cope.

Stress and loss are the most common reasons people relapse. You found something that helped you cope at a hard time — that's understandable. And now that the worst of that period has passed, you can decide what to do next.

If you're ready to try again, here's what is different this time:

- **You know you can do 3 months smoke-free** — that's proof you can do it.
- **You know one of your big triggers is stress.** Next time, you can plan ahead: who will you call, what will you do, instead of reaching for a cigarette?
- **Medication helps a lot** if you didn't use it last time. Talk to a health worker about patches, gum or tablets.

Want to set a Quit Day for the next 1-2 weeks?

Chat 7 — Family member smoking around children

Modou: My uncle smokes in the house when he visits, and my baby son is just learning to crawl. I don't want to be rude.

Your son's health comes first — being respectful to your uncle and asking him not to smoke inside are not opposites.

A way to frame it: “Uncle, I love when you visit. I have to ask one thing — could you please smoke outside, even just on the porch? The baby is breathing whatever's in the room, and I want him to be okay.”

Most people, asked kindly, will respect that. Second-hand smoke is a known cause of asthma, ear infections, slower lung development and cot death in young children. It's not a small thing.

If your uncle smokes outside but then comes back in, his clothes, his hair and his breath still carry the smoke — what's called “third-hand smoke” — and it lingers in soft furnishings. Air the room out and ask him to wash his hands before holding the baby.

Would your uncle be open to thinking about quitting himself? Often, a family member's reason — a baby, a grandchild, a sick parent — is the strongest motivation.